M23 Labetalol

Scope of Practice	EP
Classification	• Beta Blocker
Usual Presentation	• 100mg/20ml ampule (5mg/ml solution)
Mechanism of Action	• Blocks beta-1 adrenergic receptors in the heart causing a decrease in heart rate and contractility, resulting in reduced blood pressure. • Weakly blocks adrenergic receptors causing vasodilation resulting in reduced blood pressure.
Indications	• Pre-eclampsia with severe hypertension • Eclampsia
Contraindications	• Known allergy • Asthma • COPD • 2nd or 3rd degree heart block (without pacemaker) • Known sick sinus syndrome (without an internal pacemaker in place) • Patient taking non-dihydropyridine calcium channel blockers • Heart failure
Precautions	• 1st degree heart block (may cause bradycardia)
Side Effects	• Hypotension • Nausea • Dizziness • Headache

Adult Dose & Administration	**Pre-Eclampsia & Eclampsia**
	• 20mg IV/IO over 2 minutes (maximum 80mg)
	• May repeat every 10 minutes as required
	
	Dilution
	• Due to the large size of the Labetalol ampule, prepare x2 20ml syringes with the same preparation instructions:
	  • In a 20ml syringe, add 12mls of normal saline.
	  • Add 8mls of Labetalol
	  • The syringe concentration is now 40mg/20ml (2mg/ml)
	  • Administer 10mls (20mg) of the preparation over 2 minutes.

	• Not for pediatric administration
Use in Pregnancy and Lactation	• Pregnancy: Safety has not been established. Labetalol should be administered if clinically indicated. • Lactation: Infant harm is possible. If administered mother should be advised to stop breast feeding until advised by their physician.
Additional Information	• The dilution guideline above is intended to maintain cleanliness, minimize contamination risk, and eliminates the need to handle an open ampule if repeat dosing is required. • Label both syringes with “Labetalol 2mg/ml”